

OUTLINE OF EVIDENCE – MR CORY LEA SHEPHERD (APPELLANT)

Served pursuant to direction 2 of the Further Directions Order (3) dated 19 August 2026. Not filed in the Industrial Registry.

I will give evidence of the following. Paragraph references are to the notice to admit facts served 28 August 2026 (the Notice); its Annexure A holds the documents referred to.

- 1. Employment and the role.** I commenced at Logan Hospital Switchboard on 25 March 2019 as a casual, became permanent from 3 March 2021 and full time from 16 October 2023, as a continuous shift worker. The duties: the emergency response notifications, "strictly adhering to protocols and timeframes"; the pager and on-call records on which emergency paging depends; and some 200 to 300 calls a shift, including urgent clinical handover and distressed patients and families; and why an accurate pager or phone entry matters in that setting, where an urgent page goes to whatever number is recorded. My background before Queensland Health: a safety advisor on Manus Island, Papua New Guinea, and before that a third-year Bachelor of Nursing student, which is the background on which I raised the matters below as safety matters. [Notice ¶¶ 1–16, 26–38]
- 2. Sleep and fatigue.** My sleep on rotating shifts across 2023 and 2024 and the melatonin and temazepam prescribed on 16 November 2023. The shifts of 17 and 18 March 2024: my travel time each way, finishing at 23:00 and starting at 06:00, the rest and sleep actually available, the emergency codes I handled on 18 March, and the sick leave of 19 March; that those shifts were not a staff-initiated swap, the only case in which, by Ms Forrest's email of 7 July 2026, the 8-hour agreement applied. The fatigue concerns I raised; the fatigue toolkit at clause 10.4.1 of the Operations Manual, which gives management the tools to roster for and manage fatigue and was not in use at the Switchboard, which I sent to Ms Taylor and had no response; my request of 8 April 2024 and the refusal of 1 May 2024; and that before 30 June 2024 I had no fatigue training, no fatigue or psychosocial risk assessment of my rostering, and no fatigue management process. [¶¶ 17–25, 211–256, 263–268]
- 3. Rostering: lates then earlies.** The pattern on my roster line of late shifts followed by early shifts, and of nights followed by days off and then nights again, which I raised on 16 April 2024; that rostering errors on my line recurred across successive roster periods; and Ms Reese's acknowledgement of 10 May 2024 of "a few rostering errors made by Chloe with regards to Cory's line in past rosters". [¶¶ 211–223]
- 4. Pay and entitlements.** The public holidays I was not rostered between February and April 2024 and the difference to my pay. The loop: on 8 April 2024 I asked Ms Taylor to review my pay; on 9 April she directed me to raise the older dates through MyHR payroll enquiries; Payroll told me the payments had to be processed by my manager, which I relayed to her on 24 April; on 3 May Payroll wrote to her, copying me, to "submit an AVAC to correct these shifts for each fortnight"; when I followed up with Payroll on 10 May, Payroll on 13 May told me to "speak to your Line Manager"; on 21 May she was still "waiting payroll confirmation"; on 28 May I was asked to sign a validation of claims older than three months. That under the payroll process only my manager could submit the AVAC. The February 2024 special pandemic leave application: what I attached, the two declines, and the approval on the same material. [¶¶ 114–142, 182–210, 242–246]
- 5. The database, the misdirected calls, and what I could not fix.** That from 18 July 2023 operators could no longer correct database entries and the Contact & Number Changes book was removed, so a wrong number waited on Ms Stibbard or Ms Taylor and after hours had to "wait until either Chloe or myself are back"; the directives of 15 April, 19 April and 9 May 2024, and that I was not consulted before any of them. That from 15 April 2024, when Ms Taylor notified the department that she and Ms Stibbard had placed themselves on after-hours call, departments and clinicians ringing during my shifts with pager and phone updates asked for her, and on many occasions neither they nor I could have the update or issue resolved during the shift. What I saw at the console between 2 and 8 May 2024: the nine occasions the MASPER Registrar reported of calls reaching the wrong team, including the MET call team ringing because "switchboard could not tell them where VHUB was"; the Integrated Respiratory Service writing on 15 and 20 May 2024 that "we can not help patients or other clinical staff"; that on each occasion I could not correct the entry myself and the delay built up while the correction waited; and Ms Taylor's statement to the department on 9 May 2024 of "many ongoing issues raised by the MASPER and the medical department about calls being transferred to the wrong medical teams". [¶¶ 39–113]
- 6. Ms Taylor's hours: the question, the retraction request, and the answer.** That neither I nor the department knew her office hours; my email of 15 May 2024 at 1:15 pm asking her to share them and that changes be made in consultation with the team; Ms Reese's reply that evening asking me to retract it, which I did; that two days later, on 17 May 2024 at 9:30 am, Ms Taylor sent the whole department an email headed "Switchboard Manager - On call and Hours." stating "Otherwise my hours are from 06:30-14:30", the very information I had asked for; and Ms Reese's reply of 21 May 2024 asking me to identify the directives I was concerned about. [¶¶ 70–88]
- 7. The book, the earlier concerns, and the delegate role.** The communication book entry of 6 June 2023 and its removal; the concerns I raised on 7 August 2023 and the responses; my complaint of 13 May 2024, later determined to be a public interest disclosure (content not set out); and my interest in becoming the Switchboard union delegate and what followed. [¶¶ 143–181, 283–299]
- 8. My health before June 2024.** That before 18 June 2024 I had never been diagnosed with or treated for depression. My general practitioner's record of 16 November 2023 notes poor sleep with shift work, that I could not do shifts without a good sleep, no psychological illness such as depression or psychosis, and mood good. The ADHD and anxiety noted in my history in October 2022 and the specialist referral renewed on 16 May 2024, and what each concerned. [General-practice records disclosed by the Respondent]
- 9. Onset, presentation and diagnosis.** My symptoms from about 18 June 2024 and their effect on sleep, concentration, self-care and relationships. Attending Dr Slawinski on 28 June 2024, whose note records "stress at work" and "upset by people not following rules", and Dr Hawes on 1 July 2024, whose note records "work stress", that "they withhold pay at times, no overtime- not processed, manipulate his roster- so he works lates then earlies". Lodging my application for compensation on 1 July 2024. Dr Hawes's certificates of 1 July and 8 September 2024 and his psychiatrist referral. My first consultation with Dr Krishnaiah on 24 October 2024, on that referral, when I was told I had major depressive disorder with anxious distress and was started on fluoxetine; the later increase in dose and the addition of quetiapine at night; his written report of 13 February 2025; and my treatment since. The matters at paragraphs 2 to 7 above are the matters I reported to each doctor, as their records show. I offer no clinical or diagnostic opinion.
- 10. After 18 June 2024, and documents.** Meetings scheduled while I was on certified leave; that my employment was ended under abandonment provisions in October 2024 while I held current medical certificates, and my later reinstatement; my return on reduced hours and the leave taken since; that my Queensland Health email account was restricted, so I cannot produce my work emails myself. I will produce the rosters and payslips, my emails and messages with my line manager and the Director, the leave and payroll records produced by the employer, the role description, the medical records referred to above, and the letter of Metro South Hospital and Health Service of 5 June 2026 (ref K-LM26/729).

This outline states the topics on which I will give oral evidence. It is not a statement of evidence and is not verified.